SACTCHECK · REGIMEN-SPECIFIC SACT CONSENT

PROTOTYPE · GOVERNANCE REVIEW REQUIRED

Apalutamide: prostate cancer with ongoing ADT

High-risk nonmetastatic castration-resistant or metastatic hormone-sensitive prostate cancer · NCCP 00574 · version 5 · 28-day cycle

Patient name	DOB	Hospital no.	Date

Treatment context
This treatment aims to control prostate cancer and delay progression. The expected benefit depends on your disease setting, previous treatment and general health.

Schedule
Apalutamide 240 mg once daily, continuously while helping and tolerated, with ongoing ADT unless surgically castrated. Lower prescribed doses are 180 mg or 120 mg daily when indicated.

1. TREATMENT DISCUSSION

- ☐ Purpose, expected benefit and alternatives explained
- ☐ Indication, medicines, sequence and duration confirmed
- ☐ Blood tests and any individually indicated heart checks discussed
- ☐ Support medicines and phase changes explained
- ☐ Fertility, contraception and breastfeeding discussed where relevant
- ☐ Common, serious and lasting drug-specific risks discussed below
- ☐ Urgent symptoms and local contact details confirmed

2. REGIMEN / AGENT-SPECIFIC MATERIAL RISKS

Apalutamide

ANDROGEN PATHWAY TREATMENT

- ☐ Rash, including rare severe blistering or systemic skin reactions
- ☐ Fatigue, hot flushes, falls and fractures
- ☐ Underactive thyroid, weight loss, diarrhoea and joint pain
- ☐ Seizures and ischaemic heart or cerebrovascular events
- ☐ Rare lung inflammation, which can be severe or fatal
- ☐ Strong interactions with other medicines; sexual or fertility effects alongside ADT

Discuss the treatment aim, setting-specific evidence, alternatives and declining treatment; oral medicine and ADT risks, bone and cardiovascular health, sexual function, fertility, contraception, interactions, required tests and urgent contacts. Report rash promptly. Blisters, peeling skin, fever with rash, or mouth/eye sores need immediate medical advice. New or worsening cough or breathlessness requires urgent assessment for lung inflammation. Seizures require emergency help. Discuss driving and medicine interactions.

Individual aim / benefit, alternatives, material risks and questions:

3. CONSENT CONFIRMATION

- ☐ Expected benefit, important risks and reasonable alternatives discussed
- ☐ Patient questions invited and answered; patient-specific material risks / priorities considered
- ☐ Emergency contact / toxicity advice and written information offered where appropriate
- ☐ Patient understands consent is voluntary and may be withdrawn

Patient / person giving consent _____ Date _____

Clinician obtaining consent _____ Date _____

Prototype only. This one-page form is a regimen-specific consent aid and requires formal governance, consultant and oncology-pharmacy review before clinical use. The current NCCP regimen, local/national consent policy, current medicine information, patient-specific material risks and clinical judgement remain authoritative.
Base regimen: NCCP 00574 v5.

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